Ysbyty Gwynedd
Anaesthetics Department
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PERI-OPERATIVE

YG CG 414 V 1 JULY 2021

Anaesthesia for Colorectal Surgery

REVIEW BY JULY 2024

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Reviewer: Anaesthesia Department

It is the responsibility of the prescriber to ensure that there are no contraindications to the following:

Premed:

'Pre-Op' carbohydrate drinks on evening prior and on morning of day of surgery (can be taken up to 2 hours prior to surgery, followed by sips of water (no more than 50ml / hour) for the 2 hour period immediately prior to theatre)

Anaesthetic:

Spinal anaesthesia using 1 - 2ml 0.5% Hyperbaric bupivacaine + 0.4 – 1mg diamorphine (10mcg/Kg **lean body weight** up to 80 years of age)* (unless contraindicated)

* Beware of an increased incidence of pruritus if using higher doses of intrathecal diamorphine, and prescribe naloxone & chlorphenamine PRN.

+ Abdominal wall regional anaesthetic blocks e.g. TAP block (4 quadrant if incision above T9) or rectus sheath block with catheters inserted for postoperative top ups (midline incisions only)

OR

Thoracic epidural catheter insertion (particulary recommended for a high incision laparotomy (above T9) in patients with significant respiratory disease (unless contraindicated)).

Pre-incision:

Antibiotics (usually Gentamycin 240mg iv (or 160mg if <60Kg) & Metronidazole 500mg)

Dexamethasone 0.1mg/Kg

Intra-op:

Avoid nitrous oxide

Use short acting volatile agents (alternatively consider using Total Intravenous Anaesthesia)

Maintain adequate muscle relaxation during surgery to optimise surgical access (consider continuous TOF monitoring)

Maintain normothermia

Paracetamol 1g (or 15mg/Kg <50Kg)

NSAID if no contraindications *(see below)* : (iv diclofenac 75mg, or ketorolac 30mg at the end of surgery)

Opioid boluses as required (not usually required if intrathecal diamorphine has been used)

Ondansetron 4mg iv

Goal directed fluid therapy using : Cardiac output monitor (e.g ODM+) or % change in SPV/PPV(if art. line present)

Rectus sheath block with catheter insertion if an an epidural catheter not sited by the anaesthetist.

Optional: (Consider one or more of these if administration of intrathecal diamorphine is not possible)

- Pregabalin (as premedication) 150mg PO 1 – 4 hours pre-incision (75mg if elderly, <60Kg or eGFR < 30
- Low dose Ketamine 0.15mg/Kg iv pre-incision *(NB Ketamine comes in 2 different concentrations. 100mg/ml concentration must be diluted – take care not to get them confused)*
- Magnesium Sulphate 20mmol infusion during surgery titrate rate to HR and BP.

Post-op: ***Call Acute Pain team (bleep 115) or Anaesthetist (063) if pain needs reviewing urgently***

Morphine PCA (1mg bolus, 5 minute lockout)

+ Rectus Sheath Infusion (8mls /hr of 0.125% Levobupivacaine via elastomeric pump) (for midline incisions)

OR

Epidural infusion

Plus:

Regular : Paracetamol 1g (or 15mg/Kg if <50Kg) QDS

Ondansetron 4mg iv / po TDS for 48 hours

(Caution with NSAIDS postoperatively due to a possible link with anastomotic dehiscence – check with surgeon before prescribing)

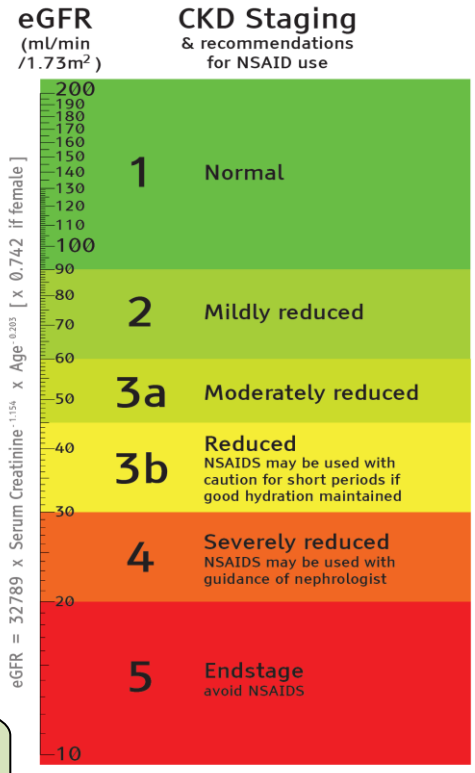
PRN : Cyclizine, Naloxone, Chlorphenamine

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To be prescribed by surgical team:

Post-op antibiotics

Clexane 40mg s/c in evening (20mg if <50Kg, 40mg BD if >100Kg) (can be commenced from 4 hours post spinal anaesthesia)



Estimated GFR is shown on the U&E